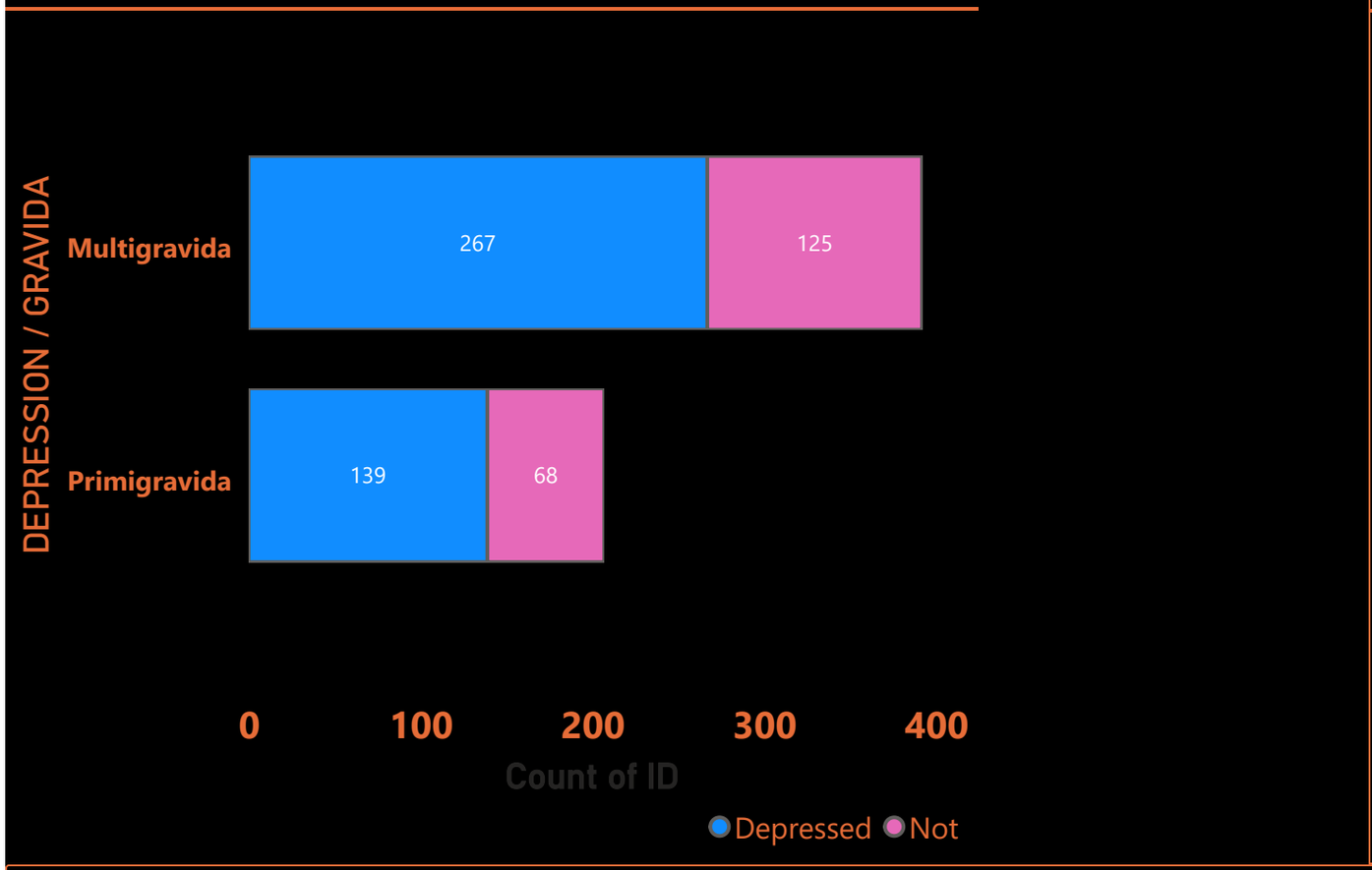
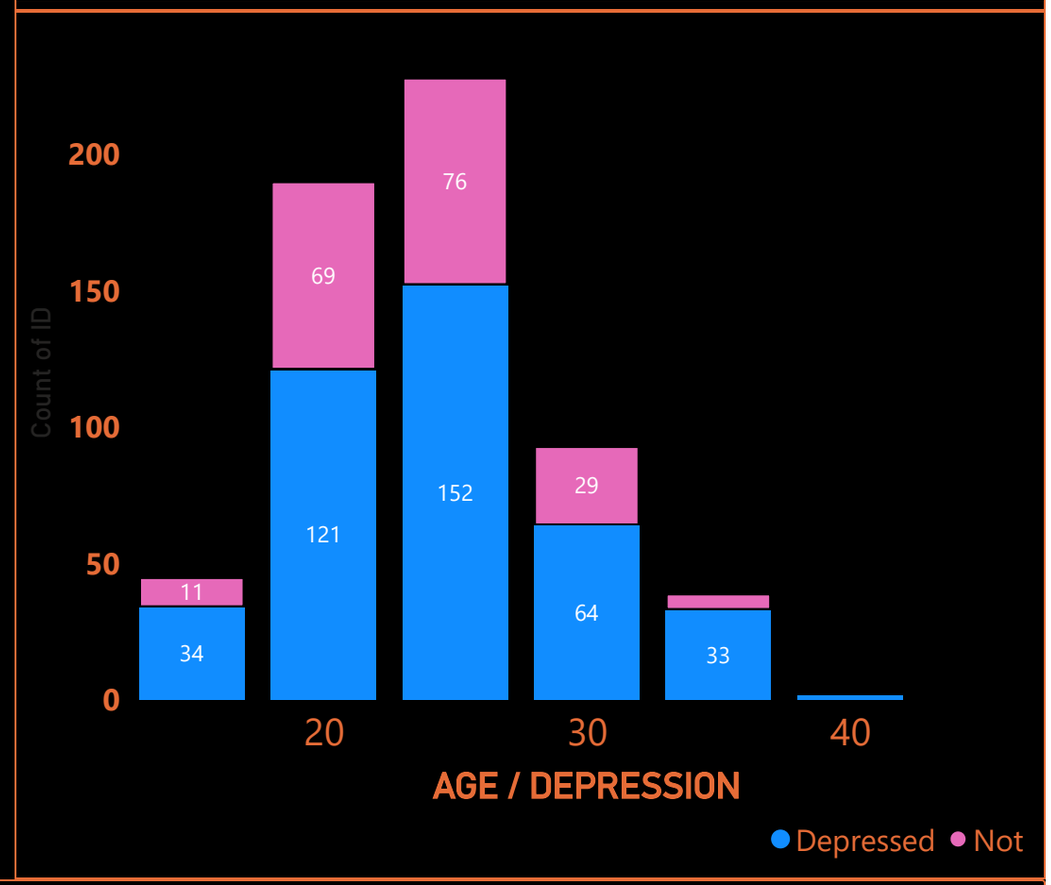
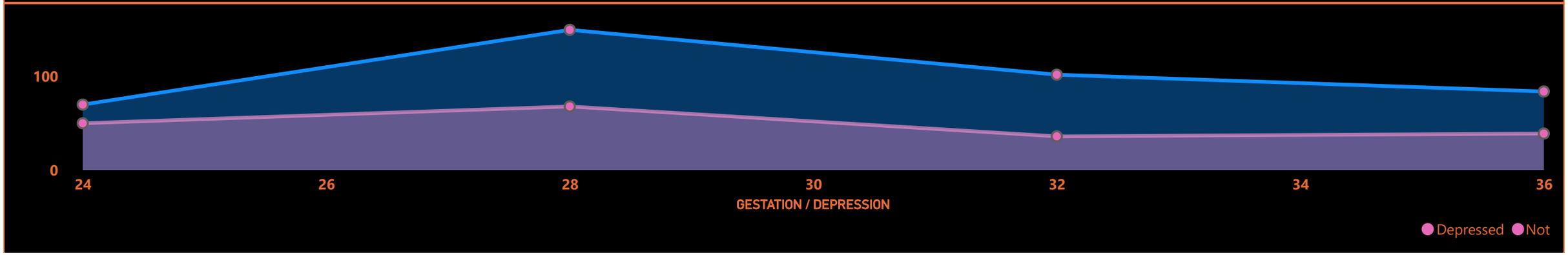
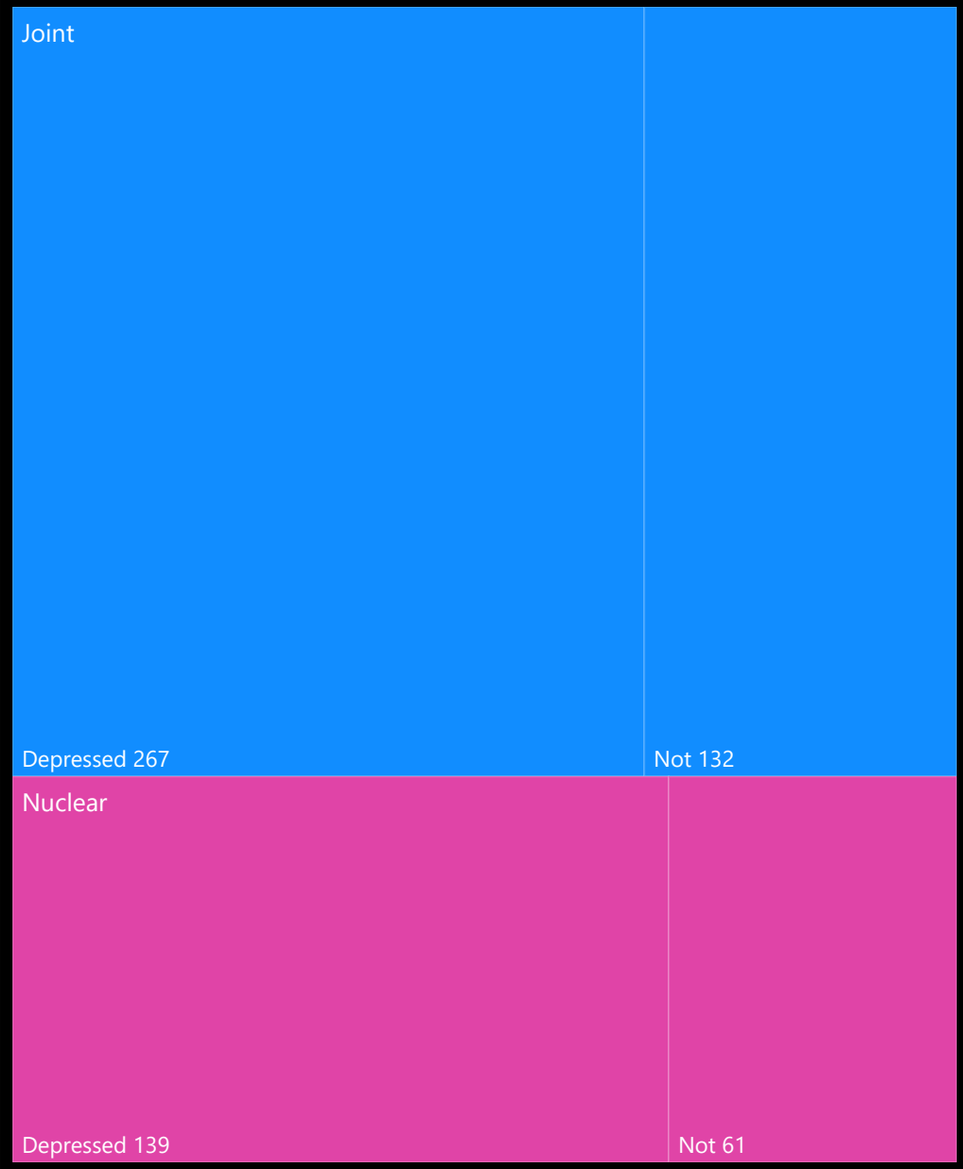




Gestation /depression



FAMILY STASTUS / DEPRESSION

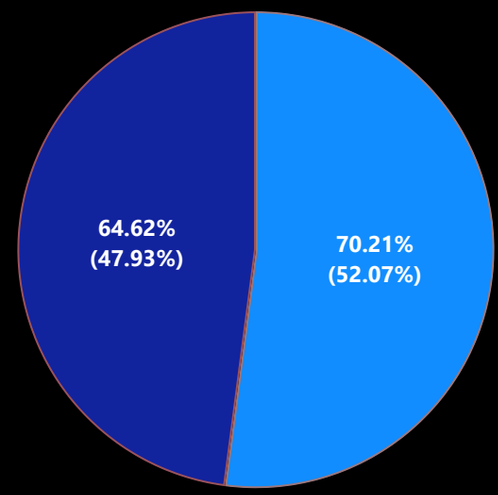


Family System ● Joint ● Nuclear

67.78%

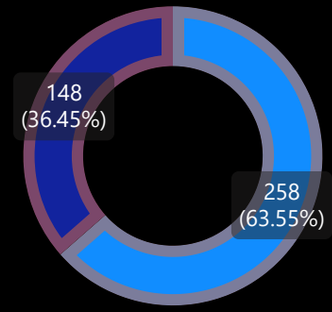
Depression %

POVERTY / DEPRESSION

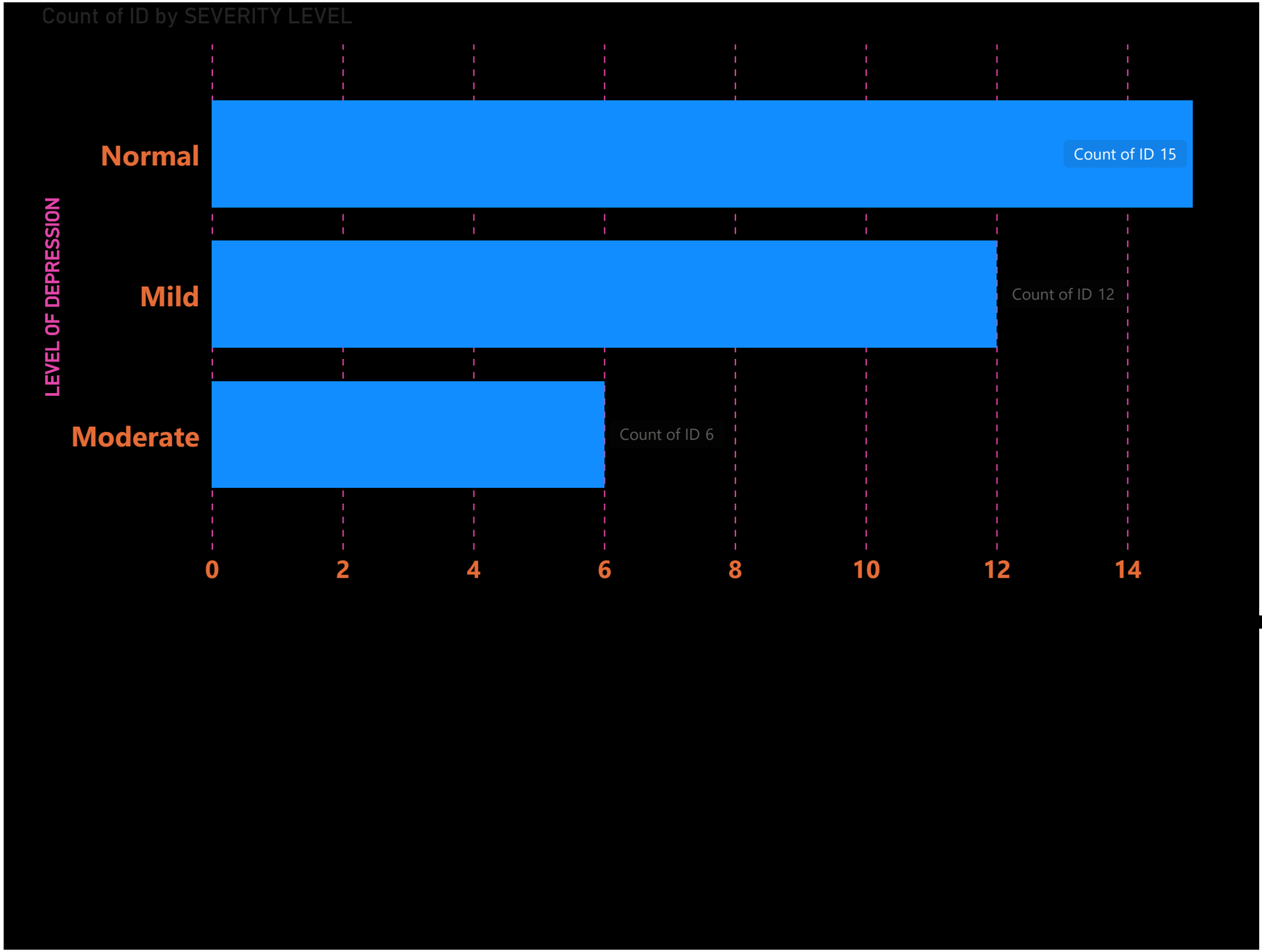


Poverty Line ● Below ● Above

MISSCARRIAGE /DEPRESSION



Previous Misc... ● No ● Yes



AGE GROUP

15 | 20 | 25

30 | 35 | 40

45

Poverty Line

Above | Below

Family System

Joint | Nuclear

Perinatal Depression Analysis

Overview

This study analyzed data from 600 pregnant patients to identify the primary drivers of perinatal depression using the EPDS framework. My findings indicate that 68 % of the sampled population experienced significant depressive symptoms, highlighting a critical health concern.

- *Socio-Economic Impact*: There is a significant correlation between financial status and mental health, with women Below the Poverty Line (BPL) showing higher rates of depression.
- *Family Structure*: Interestingly, women living in Joint Family systems reported higher levels of stress compared to those in nuclear families, likely due to increased household responsibilities and reduced privacy.
- *Gestational Peak*: A critical finding is that depression symptoms peak around the 28th week of pregnancy, identifying this as the most vulnerable period for clinical intervention.

DETAIL ANALYSIS & METHDOLOGY

1.Health& Depression

a)gravida stastus

- Horizontal Clustered Bar Char
- multi-pregnant women exhibit a significantly higher prevalence of depression compared to primigravida (first-time) mothers.

b)maternal age

- Stacked column chart
- depressive symptoms are most frequent in the 25–29 age group.

c)gestational age

- Area chart
- preterm delivery stages correlate with an increased risk of depression.

2.Socio demograhic

a)family system(nuclear /joint)

-

CONCLUSION

This analysis of 600 patients confirms that perinatal depression is heavily influenced by socioeconomic and household factors. The data highlights that women in joint family systems and those facing financial instability (below the poverty line) are at the highest risk. Furthermore, the identification of a symptoms peak during the 28th week provides a clear timeline for healthcare providers to offer targeted psychological support.

Uses

- **Early Risk Identification:** Helps doctors identify high-risk mothers based on their socioeconomic profile before symptoms become severe.
- **Resource Allocation:** Assists hospitals in planning mental health screenings specifically around the 28th week of pregnancy.
- **Policy Advocacy:** Provides data-driven evidence to support the need for better mental health resources for families in lower economic brackets.
- **Clinical Training:** Serves as a visual tool for medical students and staff to understand the multi-faceted nature of depression in maternal health.